

Utilization of a health-system specialty pharmacy to optimize management of HCV-positive kidney transplant recipients

Sandra Poon, PharmD; Casey Fitzpatrick, PharmD; Ana Lopez Medina, PharmD, PhD; Andrew Wash, PharmD, PhD; Jessica Mourani, PharmD



Background

- Over 100,000 patients in the U.S. are awaiting a kidney transplant, and the average wait time is approximately 3 to 5 years.¹
- Historically, individuals living with hepatitis C virus (HCV) have been underutilized as donors due to concerns related to disease transmission.²
- Advances in HCV treatment have led many centers to consider these individuals as donors to help reduce transplant wait times.²
- Health-system specialty pharmacy (HSSP) teams can play an important role in the kidney transplant process to improve medication access by addressing cost barriers for direct-acting antivirals (DAAs) and minimizing delays in therapy initiation.

Objective

To describe the development of an HSSP program for the management of kidney transplant recipients receiving a kidney allograft from a donor with a positive HCV nucleic acid test (HCV-NAT+).

Description

Quality Improvement Initiative

This quality improvement initiative was conducted at a health system located in Portland, Oregon from November 2024 to May 2025.

Evaluation Measures

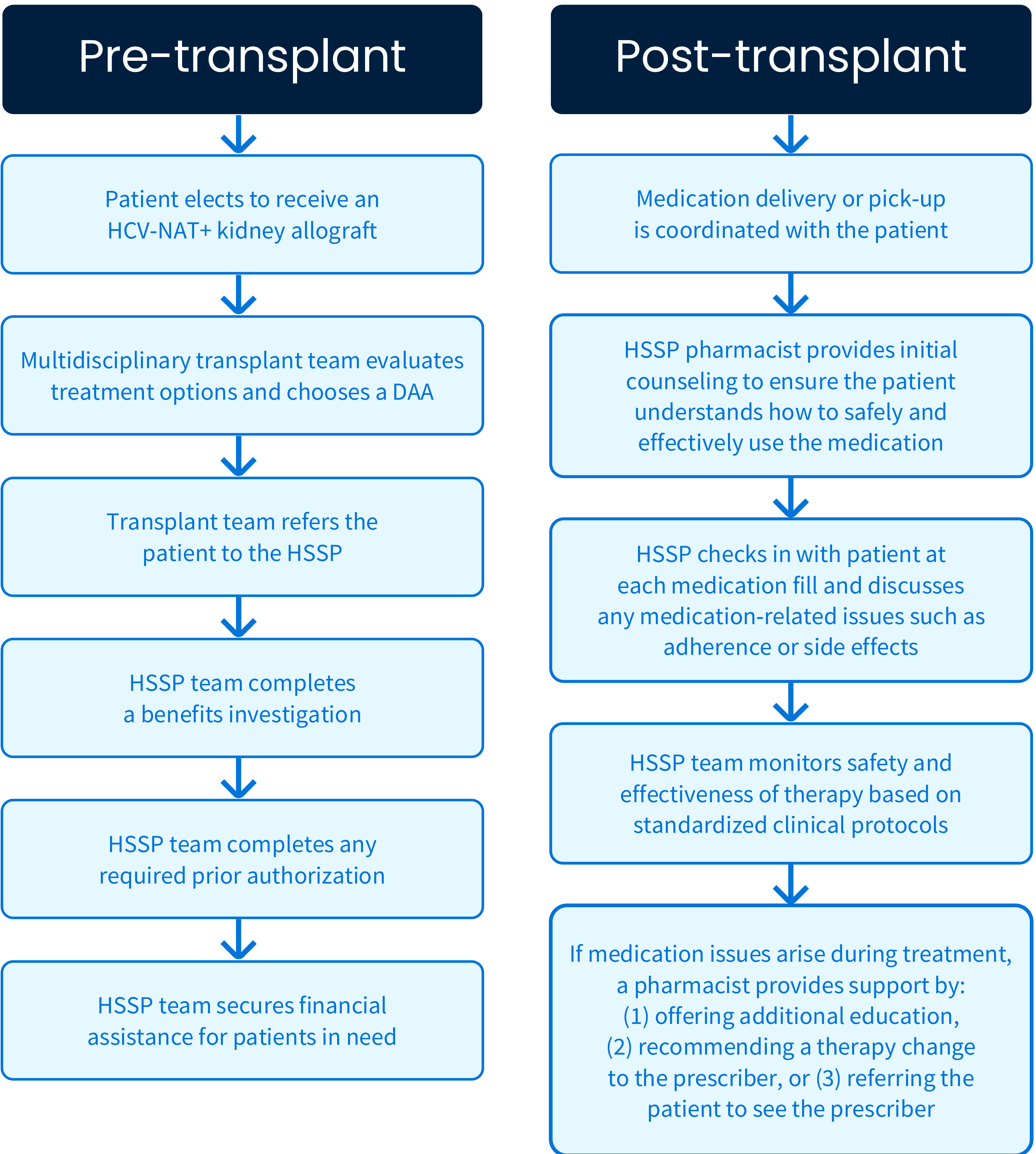
- Number of patients who elected to receive an HCV-NAT+ kidney allograft and were referred to the HSSP for DAA initiation
- Prior authorization approval rate
- Number of patients receiving financial assistance
- Mean time to treatment initiation

Data Analysis

- Descriptive statistics were used to present all measures.

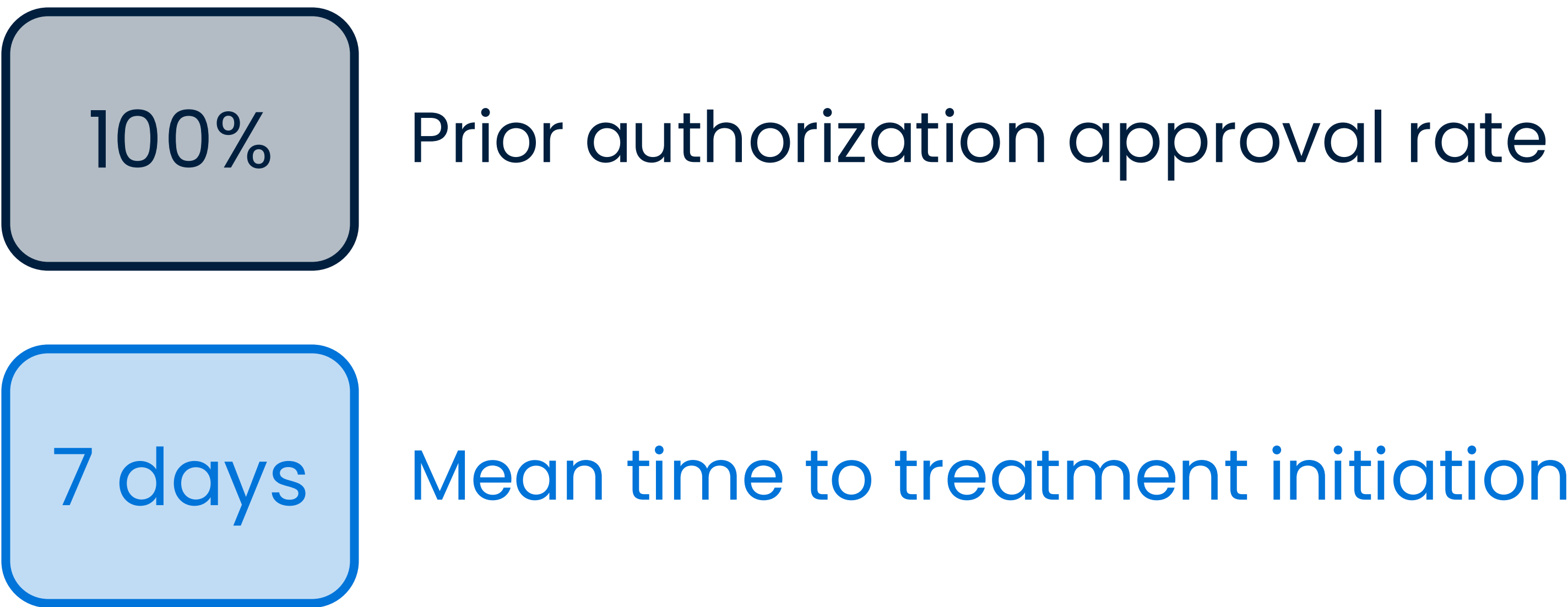
Description

Figure 1: Workflow for Patients Receiving an HCV-NAT+ Kidney Allograft



Evaluation

To date, three patients who elected to receive an HCV-NAT+ kidney allograft have been referred to the HSSP for DAA initiation. The HSSP team successfully secured financial assistance for two patients.



Discussion & Conclusion

- Given the demand for kidney transplants far exceeds the available organ supply, utilizing HCV-NAT+ donors can help expand the donor pool and shorten wait times for recipients.
- HSSP teams have expertise in supporting medication access and managing complex conditions.
- HSSP teams can be valuable assets in the clinical management of HCV-NAT+ kidney transplant recipients.

Next Steps

- Future studies should evaluate clinical outcomes for patients receiving HCV-NAT+ kidney allografts who initiate DAAs, including the impact of treatment delays.

References

¹ Organ Procurement and Transplantation Network (OPTN) and Scientific Registry of Transplant Recipients (SRTR) OPTN/SRTR 2013 annual data report. Rockville, MD: Department of Health and Human Services, Health Resources and Services Administration. *Am J Transplant*. 2015 Jan;15(S2):4–15.

² Patnaik R, Tsai E. Hepatitis C virus treatment and solid organ transplantation. *Gastroenterol Hepatol (NY)*. 2022;18(2):85–94. PMID: 35505819